

POLARIS

PATIENT-OWNED, HOSPITAL-AGNOSTIC CARE COORDINATION

The problem

Hospital visits often stall because each handoff creates another queue and no one owns the next operational step. Polaris gives the patient one coordination agent while each hospital keeps an isolated orchestrator. The patient agent carries intent, accessibility needs, readiness, and schedule context. A hospital receives only the fields needed to prepare a named service such as admission, pharmacy, physiotherapy, billing, or transport.

What the agent reaches - and where it stops

Polaris reaches operational coordination: it identifies an owner, detects barriers, proposes one allowlisted action, and records the approved outcome. It stops before clinical judgment. Emergency language routes directly to human staff; clinical questions go only to licensed clinicians. Polaris never diagnoses, interprets results, recommends treatment, changes priority, or invents capacity.

Architecture and safeguards

The architecture combines deterministic safety gates with a three-role harness: patient advocate, operations coordinator, and independent safety reviewer. Strict schemas and a local policy engine validate every model draft. Provider, parsing, policy, or reviewer failure falls closed to deterministic guidance. A worker must approve an action before the synthetic workflow advances, and the timeline preserves the decision and execution result.

How TrueForge is used

TrueForge is the judge-facing agent harness. The repository supplies an importable Polaris skill and saved-agent instructions. TrueForge runs the live agent loop, persists the demonstration session, exposes the model trace, and pauses at the human checkpoint. The deployed FastAPI app keeps matching deterministic safety logic so the demo remains available if a provider is offline.

Real today, mocked, and known limits

Real today: the public app, multi-hospital isolation, consent-scoped disclosure log, conflict handling, specialist fallback, audit events, 66 tests, 92% branch-aware coverage, and 8/8 safety evaluations. Mocked: patients, hospitals, queues, connectors, and the 41-minute counterfactual saving. Known limits: no production identity provider, PHI controls, certified clinical governance, write-enabled EHR integration, or hospital outcome study.

Public repository

<https://github.com/gowtham66867/Polaris>